

CHILD ABUSE AND NEGLECT

SINU K A
PART 1
190020424

CONTENTS

- Introduction
- Definitions
- Types of child abuse & neglect
- Physical child abuse
- Shaken baby syndrome
- Battered child syndrome
- Child sexual abuse
- Child neglect
- Munchausen syndrome by proxy
- Role of pedodontist in child abuse & neglect
- Intervention & Prevention
- Legal aspects
- Conclusion
- References

INTRODUCTION

- Childhood should be a carefree time of life filled with love, new world to explore & with joy of mastery of oneself & the environment.
- However, for many children, this is only a dream, not reality.
- Child abuse & neglect is an increasing social problem not limited to medical, legal or social service professions.
- The dentist treating the children must also be able to detect, document, report & often help to manage these needy patients and their families.



HISTORICAL BACKGROUND

- First documented and reported case of CA/CN occurred in 1874 with a named MARY ELLEN.
- During the 19th century, child labour laws were passed in most industrialized countries to limit the kinds of jobs children could do and the number of hours they could work.
- 1946: In a classic article by Caffey, some common features of CA/CN were first described, and it reported the common association of subdural hematomas and long bone pathosis.
- 1962 : Term 'Battered child syndrome' by Henry Kempe
- 1974 : Child Abuse Prevention and Treatment Act
- 1972 : Kempe founded 'Kempe Centre'
- 1978 (Mclain) : coined CAN (Child Abuse and Neglect)

DEFINITIONS

Gill(1968): It is defined as the non accidental physical injury, minimal, or fatal, inflicted upon children by persons caring for them. It is an over Act of Commission of a caretaker— physical, emotional, or sexual.

Battered baby: A child who shows clinical or radiographic evidence of lesions.

WHO(1992): ‘Child abuse and maltreatment constitutes all the forms of physical and/or emotional ill-treatment, sexual abuse, neglect or negligent treatment or commercial or other exploitation, resulting in actual or potential harm to the child’s health, survival, development or dignity in the context of a relationship of responsibility, trust or power’.



PREDISPOSING FACTORS

1) Parental characteristics

- Violence
- Poverty
- Parental history of abuse
- Socially isolated
- Low self esteem
- Less adequate maternal functioning



2) Child characteristics

- Unwanted or unplanned child
- Number of children in the family
- Child's temperament
- Position in the family
- Additional physical needs if ill or disabled
- Activity level or degree of sensitivity to parental needs



3) Environmental characteristics

- Chronic stress
- Problem of divorce
- Poverty
- Unemployment
- Poor housing
- Frequent + relocation
- Alcoholism
- Drug addiction



TYPES OF CHILD ABUSE AND NEGLECT

1. Physical abuse

- Sexual abuse
- Failure to thrive

2. Intentional drugging or poisoning

- Munchausen syndrome by proxy
- Health (medical) care neglect
- Dental neglect
- Safety neglect

3. Emotional abuse and neglect

- Physical neglect

4. Fabricated or induced illnesses

- Societal abuse
- Abuse by witnessing intimate partner violence (IPV)

PHYSICAL CHILD ABUSE

- Non-accidental trauma is one of the most common types of child abuse
- Physical abuse is the most important subtype of child maltreatment

HISTORY

- Correct questions to be asked
- Eyewitness history: Usually has three aspects
 1. Child himself states that injury is caused by parent.
 2. One parent accuses the other about the injury.
 3. Parent accepts that one of the many injuries is caused by him but not all.



- Unexpected injury:
 - Dental
 - Vague explanation
 - No explanation
 - Inconsistent explanation
- Implausible history:

Many parents offer an explanation for the injury, but one which is implausible and inconsistent with common sense like describing a minor injury whereas the marks on the child prove otherwise.

- Alleged Self–inflicted injury :

An alleged self–inflicted injury in a small baby is most serious. In general, if a child cannot crawl, he cannot cause self–injury

- Delay in seeking medical case :

- Most non abusive parents seek, immediate care, when their child is injured.

- Some abused children are not presented for care for a considerable length of time even in major injury.

- Abusive parent will be that he will not accompany the child to the health care facility.

BRUISES

- Inflicted bruises :
 - Occur at typical sites or fit in recognizable patterns.

Typical sites for inflicted bruises :– Buttocks & lower back (patting)

- Genitals & inner thighs
- Cheek (slap marks)
- Earlobe (pinch marks)
- Upper lip and frenum (forced feeding)
- Neck (choke marks)

- Accidental bruises :
 - Bluish discolorations of the skin are not bruises.
 - Accidental bruises mostly lie over bony prominences whereas the abuse marks are on soft tissues.



- Unusual bruises :
 - Common ethnic practices can desult in bourses that should not be confused with child abuse.
 - The Vietnamese can induce symmterical, linear bruises, from coin rubbing. For symptoms of fever, chills or headache, the back and chest are covered with oil and then massaged in downward strokes with the edge of a coin
- Pseudo bruises :
 - Some skin conditions like Mongolian spot or allergic penorbital discolorations, Haemophilus influenzae may give appearance abusive marks.

VARIABLES AFFECTING THE APPEARANCE OF BRUISES

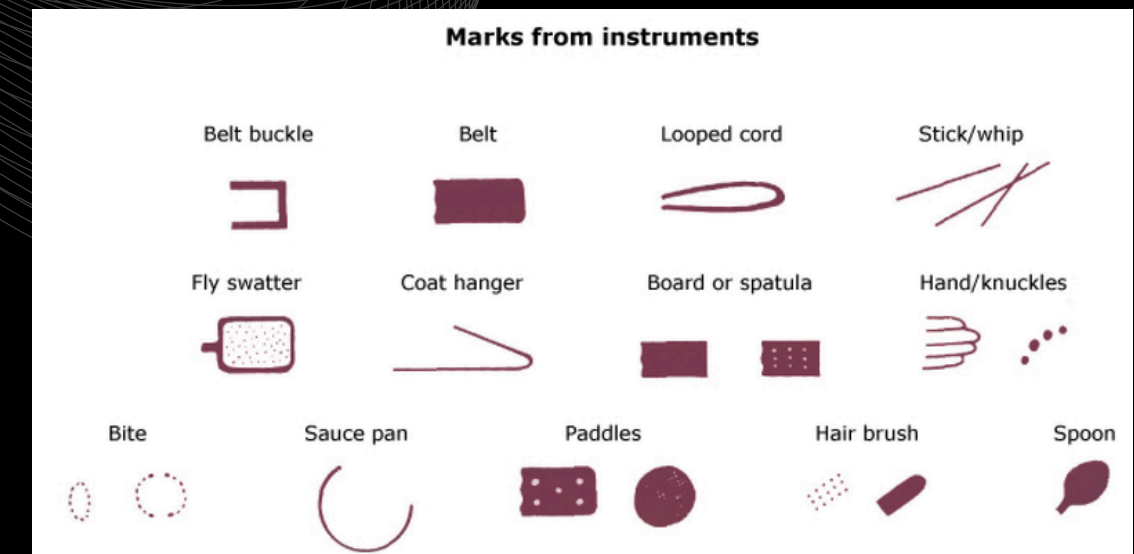
- Vascularity of the tissue injured
- Age
- Metabolic rate
- Medications
- Normal skin colour
- Mass and velocity of the impact
- Time of injury
- Other factors that affect bruising

TYPES OF PHYSICAL ABUSE

Location of bruise	Indicative of bruise
• Genital or inner thigh • Cheeks • Earlobes • Upperlip / labial frenum • Neck • Circumferential bruises on ankles / wrists • Corners of mouth	• Toilet mishaps or sexual abuse • Slapping of child • Pinching or pulling • Impatient or forceful feeding • Strangulation • Placement of restraints • Gagging of child

MARKS IN PHYSICAL CHILD ABUSE

- Human hand marks :
 - Grab marks : Oval-shaped bruises, resemble fingerprints due to holding of child in violent shaking
- Strap marks :
 - 1–2 inches wide, sharp bordered, rectangular bruises of various lengths, caused by a belt.
- Lash marks :
 - Narrow, straight, edges bruises or scratches caused by thrashing with tree branch
- Bizarre marks :
 - When a blunt instrument is used in punishment.



- Loop marks :
 - Secondary to being struck with a doubled-over lamp cord, rope, or fan belt.
 - The distal end of the loop strikes with most force, commonly breaking the skin & leaving loop-shaped scars.
- Circumferential the marks :
 - If a narrow rope or cord is used, child will be left with circumferential cut.
 - If a strap or piece of sheet is used, presenting as a large blister that encircles the extremity.
- Gag marks :
 - Seen as abrasions that appear near thr corner of the mouth.
 - Children may be gagged because of screaming or yelling.



ORAL MANIFESTATIONS OF CAN :

- Lips ;
 - Bruises
 - Lacerations
 - Scars from persistent trauma
 - Burns caused by hot food or cigarette
 - Bruising, scarring or erosion at corner of mouth



- Teeth :
 - Fractured
 - Displaced
 - Mobile
 - Avulsed
 - Non vital & darkened
 - Multiple residual roots with no plausible history to account for the injuries.
 - Unaccountable malocclusion



- Maxilla / Mandible :
 - Fracture of maxillary bone
 - Mobility or displacement of maxillary teeth
 - Luxation injuries
 - Palatal fractures
 - Bruising / swelling
 - Malocclusion due to healed fractures
 - TMJ pain or dysfunction
 - Difficulty in mouth opening
 - Deviation of mandible on opening

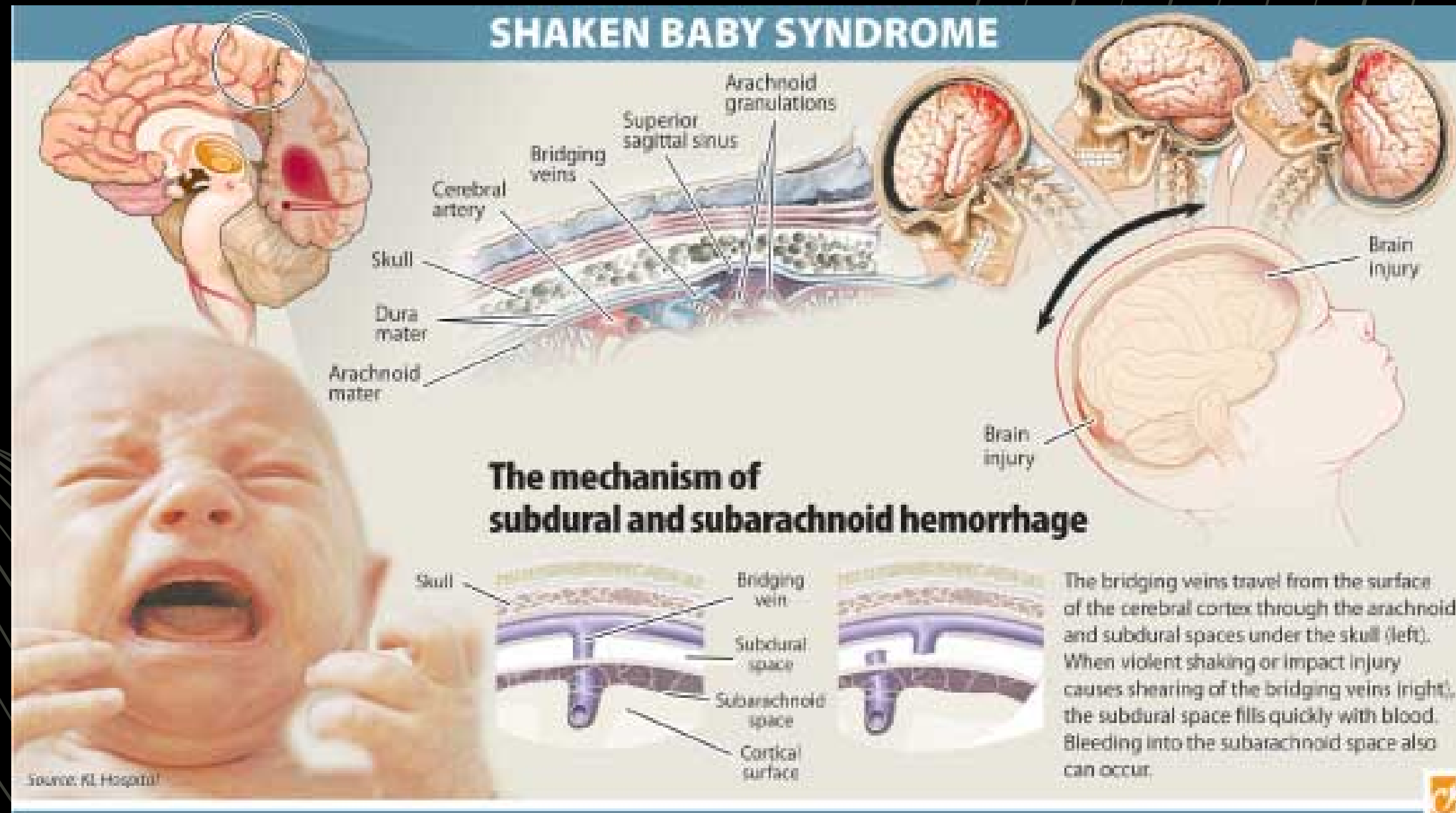
SHAKEN BABY SYNDROME

- Also called :
 - Slam syndrome
 - Shaken – impact syndrome
- John Caffey, a pediatric radiologist popularized the term ‘ Whiplash Shaken Baby Syndrome ’ in 1972, to describe clinical findings in infants that included : retinal hemorrhages, subdural and/or subarachnoid hemorrhages and/or external cranial trauma.
- Serious form of child maltreatment most often involving children younger than 2 years but may be seen in children upto 5 years.



ETIOLOGY

- Act of violent shaking leads to serious or fatal injuries.
- Generally results from tension and frustration generated by a baby's cry or irritability.



MECHANISM OF INJURY

- Whiplash forces cause subdural hematomas by tearing cortical bridging veins.

CLINICAL FEATURES

- Signs may vary from mild & non-specific to severe.
- Non-specific signs:– Moderate ocular or cerebral trauma
 - H/o poor feeding, vomiting, lethargy & irritability occurring for days or weeks.

DIAGNOSIS

- History
- Physical findings
- Radiology : CT scan & MRI
- Seen to be mildly or moderately anemic.

BATTERED CHILD SYNDROME

- (Henry Kempe, 1962) A clinical condition in young children, usually under 3 years of age who have received non accidental wholly in excusable violence or injury, on one or more occasions, including minimal as well as severe fatal trauma, for what is often the most trivial provocation, by the hand of an adult in a position of trust, generally a parent, guardian or foster parent.

BATTERED BABY

(Sewyn, 1985) is a child who shows clinical or radiographic evidence of lesions that are frequently multiple and involve mainly the head, soft tissue, the long bones and the thoracic cage and that cannot be unequally explained.

FEATURES

- Can be seen in infants through adolescents but is most common in children younger than 3 years.
- Most prevalent among stepchildren, handicapped, and first-born children.
- Victims are mostly younger than 2 years of age, with an average age of 16 months.
- Fractures are second only to soft tissue injuries as the most common finding.
- Multiple fractures in various stages of healing, corner and bucket-handle metaphyseal fractures may be a classic finding.
- Multiple bruising over the entire body, different colors shadow different dates of origin.
- Facial scarring, burns, bite marks.
- Fractured / missing teeth.
- Radiographic evidence – previous injury
- Repairing fracture lines of skull, mandible, limbs.

CHILD SEXUAL ABUSE

Factors :

- Cultural morals make sexual abuse a stigma for the victim, perpetrator, and family.
- Victims are often young children whose fears, lack of awareness, or lack of language skills make them easy prey and victims who may not be ready or believable witnesses.
- Health professionals may be unaware of the signs or symptoms of child sexual abuse.
- Child sexual abuse often is hidden with no visible physical manifestations.
- Verification of sexual abuse by physical examination may be beyond the legal extent of practice of many professionals.
- Lack of an accepted definition of sexual abuse.



VICTIM

- Sexually abused child is most often a female, ratio of 9:1
- Children of all ages are abused sexually
- Some of the features :
 - Emotional effects
 - Functional disturbances such as retention of feces
 - Frequent masturbation
 - Guilt & anxiety



PERPETRATOR

- It is no longer considered to the impersonal stranger who victimizes on unknown child.
- Most often is committed by a male parent against a female child.

ACT

- Types include molestation, intercourse, or family-related rape.
- Forms of sexual abuse may include the act of sexual abuse is rarely a singular event

ORAL MANIFESTATIONS

- Gonorrhea
- Syphilis

EMOTIONAL ABUSE

Examples:

- Verbal abuse
- Excessive demands on a child's performance.
- Discouraging caregiver and child attachment.
- Penalizing a child for positive, normal behavior.



Etiology:

- Stressful life of parents
- Drug abuse
- Alcoholism
- Divorce
- Death of a spouse
- Isolation
- Poverty
- Unemployment
- Family stress

CHILD NEGLECT

It is a form of abuse, an act of caregivers that results in depriving a child of their basic needs, such as failure to provide adequate supervision, healthcare, clothing, or housing, as well as other physical, emotional, social, educational, and safety needs.

TYPES OF CHILD NEGLECT

- Physical neglect
- Nutritional neglect
- Healthcare neglect
- Dental neglect
- Emotional neglect
- Supervisory neglect
- Environmental neglect
- Educational neglect

PHYSICAL NEGLECT

- Occurs when children's basic physical needs are not being met & often occurs in a persistent pattern.
- Abandonment, expulsion, shuttling, nutritional neglect, and clothing neglect.

NUTRITIONAL NEGLECT

- Failure to thrive can be defined as an underweight, malnourished condition who has a weight that is below the third percentile, & height and head circumference that are above 3rd percentiles on growth curves.
- On physical examination: Infants have gaunt faces, prominent ribs, wasted buttocks, & spindly extremities & is expressed in first 2 years of life.

HEALTHCARE NEGLECT

- It occurs in situations where an emergency exists, & the parents or caretakers will not acknowledge it as much.
- Refusals because of religious beliefs also lead to healthcare neglect.

DENTAL NEGLECT

- It occurs when a parent or caretaker deliberately or unintentionally permits the child to experience suffering or fails to provide the necessities for the child's physical, emotional, and intellectual development.

EMOTIONAL NEGLECT

- Defined as the continual scapegoating & rejection of child by parent or caretaker.
- Inadequate affection

SUPERVISORY/SAFETY NEGLECT

- It occurred when injury results from a lack of supervision.
- Usually involve children younger than 4 years of age.
- This leads to injuries like burns, poisonings, & falls because children are not being watched.

ENVIRONMENTAL NEGLECT

- It occurs when children's home environment are filthy.
- Related to both physical neglect and supervisory neglect.

EDUCATIONAL NEGLECT

- When children are not given access to education.

MUNCHAUSEN SYNDROME BY PROXY

- First described by Dr. Richard Asher in 1951.
- Term coined by Dr. Roy Meadow.
- Referred to as "illness induction syndrome" and "pediatric symptom falsification".
- Richard Asher reported, a parent or caretaker attempts to bring medical attention to themselves by injuring or inducing illness in their children.
- Factitious symptoms are of bleeding from various sites.
- Factitious signs include recurrent sepsis from injecting contaminated fluids, chronic diarrhea from laxatives, fever from rubbing thermometers, or rashes from rubbing the skin or applying caustic substances.

ROLE OF PEDODONTIST IN CHILD ABUSE AND NEGLECT

- Initial examination reveals trauma including oral cavity and it is within the scope of attending dentist, the definitive treatment should begin.
- To observe and examine any suspicious evidence that can be ascertained in office.
- To record according to the law any evidence which may be helpful in the case.
- To treat any dental injuries. Dentist should be acquainted with management of injuries to both primary and permanent dentitions.
- To establish & maintain a professional therapeutic relationship with the family.
- To transfer the child to a physician or hospital for proper care.



INTERVENTION & PREVENTION

PRIMARY LEVEL

- Greater attention should be given toward screening children at a higher risk of maltreatment.
- Parents at risk for abusing children are frequently very needy themselves, so they need to be screened and counseled.
- Comprehensive evaluation of the child and family situation should be done, assisted by a social worker and mental health professionals.

SECONDARY LEVEL

- The pedodontist must recognize his limitations and assume responsibilities for applying on interdisciplinary approach.
- The goal of intervention should be to enhance parenting capabilities to enable them to provide more adequate care for children and curb possible maltreatment.

TERTIARY LEVEL

- Pedodontist should ensure that child is referred to a designated child protection agency.
- He should not make the report and disengage.

LEGAL ASPECTS

- Should be fully aware of the legal standards of care and legal responsibilities.
- Records should be made in presence of patients.
- Should keep legibly written, accurate case records.
- Should keep update knowledge.
- Diagnostic tools like radiographs should always be used.
- Should always consult a legal or medicolegal expert to review insurance policies or any financial or legal matter.

CONCLUSION

- Child abuse and neglect can seriously affect a child's physical, emotional & oral health.
- Pediatric dentists are often the first to identify signs of abuse because many injuries occur in the head & oral region.
- Early diagnosis, proper documentation, & timely reporting are essential to protect the child & prevent further harm.

REFERENCES

- Textbook of Pediatric dentistry – 5th edition – Nikhil Marwah
 - Pediatric dentistry – Shobha Tandon
 - Pediatric dentistry : Scientific foundations and clinical practica : Stewart et al.
- 

THANK YOU